

Neutrophilic Dermatosis Variants and Clinical Features

Pustular Eruption and Localized Forms

Some patients previously described as having the "pustular eruption of ulcerative colitis" may be more accurately classified within a clinical variant of **Sweet syndrome**.

Neutrophilic dermatosis of the dorsal hands, also known as **pustular vasculitis of the dorsal hands**, is a localized pustular variant of Sweet syndrome characterized by lesions predominantly confined to the dorsal aspects of the hands. The lesions are morphologically similar to those seen in classic Sweet syndrome and typically resolve rapidly with systemic corticosteroids and/or dapsone therapy. Many individuals with this variant also have concurrent lesions on the oral mucosa, arms, legs, back, or face.

Subcutaneous Sweet Syndrome

The cutaneous manifestations of **subcutaneous Sweet syndrome** usually present as erythematous, tender dermal nodules on the extremities. When located on the legs, these nodules often mimic **erythema nodosum** (see Chap. 69). Given that Sweet syndrome can occur concurrently or sequentially with erythema nodosum, biopsy of new dermal nodules may be required to confirm the diagnosis—even in patients with a prior biopsy-confirmed history of Sweet syndrome. Diagnostic criteria are summarized in Table 31-2.

! [Figure 31-3: Acute febrile neutrophilic dermatosis. Typical lesion consisting of coalescing, plaque-forming papules. A. Bright-red lesions on the neck. B. Lesion on the dorsum of the right hand exhibiting the "relief of a mountain range" feature.]

Related Physical Findings

Extracutaneous Manifestations (Table 31-3)

Sweet syndrome can involve multiple organ systems beyond the skin, including the:

- Bones (sterile osteomyelitis, particularly in children)
- Central nervous system
- Ears
- Eyes
- Kidneys
- Intestines
- Liver
- Heart
- Lungs
- Mouth
- Muscles
- Spleen

Ocular manifestations—such as conjunctivitis—may be the initial presentation of Sweet syndrome. However, ocular involvement is variable in classic cases and uncommon in malignancy-associated or drug-induced forms.

In contrast, **oral mucosal involvement** (e.g., painful ulcers) is more frequently observed in patients with **hematologic disorders** and is rare in classic Sweet syndrome. Oral lesions, like other extracutaneous manifestations, typically resolve with systemic corticosteroid therapy.